



QUALITATIVE AND QUANTITATIVE COMPOSITION

Each prolonged release tablet contains Alfuzosin hydrochloride 10mg

PHARMACEUTICAL FORM

Round, biconvex 3-layer tablets: one white layer between two yellow layers

CLINICAL PARTICULARS

Therapeutic indications

- Treatment of the functional symptoms of benign prostatic hypertrophy
- Adjuvant treatment to a catheter in acute urinary retention related to benign prostatic hypertrophy

POSODOGY AND METHOD OF ADMINISTRATION

Oral use

The tablet must be swallowed whole with a glass of water (see Special warnings and precautions for use). The recommended dosage is one 10mg tablet per day, to be taken immediately after the evening meal

Adjuvant treatment to a catheter in acute urinary retention related to benign prostatic hypertrophy:

The recommended dosage is one 10mg tablet per day, to be taken after a meal, from the first day of catheterization onwards. The treatment is administered for 3 to 4 days, i.e. 2 to 3 days while the catheter is in place and one day after it is removed

CONTRAINDICATIONS

This medicinal product must not be administered in the following situations:

- hypersensitivity to alfuzosin and/or any of the other ingredients
- postural hypotension
- liver failure
- severe kidney failure (creatinine clearance < 30 ml/min)
- concomitant administration of ritonavir alone or in combination with ombitasvir / paritaprevir, lopinavir or nirmatrelvir (See Section Interactions)

SPECIAL WARNINGS AND PRECAUTIONS FOR USE

Special warnings

This medicinal product must be used with caution in patients treated with antihypertensives or nitrate derivatives.

The taking of this medicinal product with alpha-blocker antihypertensives is not recommended (see section Interaction with other medicinal products and other forms of interaction).

In some people, orthostatic hypotension may occur within hours of taking the medicinal product, possibly accompanied by symptoms (dizzy spells, fatigue, sweating). If this happens, the patient should be kept in a supine position until the symptoms have completely disappeared.

These phenomena are usually temporary, occur at the start of treatment, and do not generally prevent the continuation of treatment.

A marked decrease in blood pressure was reported during post-marketing surveillance in patients with preexisting risk factors (such as underlying heart disease and/or concomitant treatment with antihypertensive medicinal products).

There is a risk of ischaemic strokes, particularly in elderly patients with pre-existing symptomatic cerebrovascular disorders (such as cardiac arrhythmia, atrial fibrillation, history of transient ischemic attack) or asymptomatic cerebrovascular disorders, because of the onset of hypotension after the administration of alfuzosin (see section Undesirable effects).

Patients should be informed of the possibility of the onset of these events. Caution is recommended, especially in the elderly. Indeed, the risk of onset of hypotension and associated symptoms may be higher in elderly patients.

As with alpha-1 blockers, this medicine should be used with caution in patients with acute heart failure.

Patient with congenital prolonged QTc interval prolongation, a history of QTc interval prolongation, or who are being treated with medicinal products which increase the QTc interval must be monitored before and during treatment.

Combination of alfuzosin with potent CYP3A4 inhibitors (such as itraconazole, ketoconazole, protease inhibitors, clarithromycin, telithromycin and nefazodone) must be avoided (see section Interaction with other medicinal products and other forms of interaction). Alfuzosin must not be used in combination with CYP3A4 inhibitors which are known to prolong the QTc interval (for example, itraconazole and clarithromycin). If such treatment is initiated, temporary discontinuation of alfuzosin is recommended.

Rare cases of priapism (prolonged painful erection not related to sexual activity) have been reported in patients treated with alfuzosin as well as with other alpha-1 blockers. Prompt (sometimes surgical) management of patients is essential. Priapism can actually lead to permanent impotence if not properly treated.

Intraoperative Floppy Iris Syndrome (IFIS, a variant of narrow pupil syndrome) has been observed during cataract surgery in some patients treated or previously treated with tamsulosin. Isolated cases have also been reported with other alpha-1 blockers and the possibility of a class effect cannot be excluded. Given that IFIS may cause additional technical difficulties during cataract surgery, previous or current administration of alpha-1 blocker medicinal products must be reported to the ophthalmologist prior to surgery, even though the risk of onset of IFIS with alfuzosin is low.

Given the lack of data on safety in patients with severe renal failure (creatinine clearance < 30ml/min), Xatral XL 10mg, prolonged-release tablets should not be administered to these patients. This medicinal product contains castor oil and may cause gastrointestinal disorders (mild laxative effect, diarrhea).

Precautions for use

Caution is recommended when alfuzosin is administered to patients who have had pronounced

hypotension in response to the administration of another alpha-1 blocker. In patients with coronary disease, alfuzosin should not be prescribed in isolation. The specific treatment of coronary insufficiency should be continued. If angina reappears or worsens, treatment with alfuzosin should be discontinued. Use with PDE-5 inhibitors: the concomitant administration of Xatral LP 10 mg with a phosphodiesterase-5 inhibitor (for example, sildenafil, tadalafil or vardenafil) may result in symptomatic hypotension in some patients (see Section Interaction with other medicinal products and other forms of interaction). In order to reduce the risk of onset of orthostatic hypotension, patients must be stabilized on alpha-blocker treatment before initiating treatment with a phosphodiesterase type 5 inhibitor. In addition, it is recommended that treatment with the phosphodiesterase-5 inhibitor is started at the lowest possible dose.

Patients should be advised that the tablets must be swallowed whole. The tablets must not be crushed, chewed, crushed or ground to a powder. These actions may lead to inappropriate release and absorption of the medicinal product and consequently to possibly early undesirable effects.

INTERACTION WITH OTHER MEDICINAL PRODUCTS AND OTHER FORMS OF INTERACTIONS

Medicinal products causing orthostatic hypotension

As well as the antihypertensives, many medicinal products may lead to orthostatic hypotension. This is particularly the case with nitrate compounds, phosphodiesterase type 5 inhibitors, alpha-blockers used for urological purposes, imipramine-like antidepressants and phenothiazine neuroleptics, dopamine agonists and levodopa.

Using them in combination therefore risks increasing the frequency and intensity of this undesirable effect. Refer to the interactions specific to each class with the corresponding restriction levels.

Contraindicated combinations

+ Ritonavir alone or in combination with ombitasvir/paritaprevir, lopinavir or nirmatrelvir (see section Contraindications).

Increased plasma concentrations of alfuzosin due to a decrease in its hepatic metabolism by the dual therapy.

Combinations not recommended

+Alpha-blocker antihypertensives (doxazosin, prazosin, urapidil)

Increased hypotensive effect. Risk of severe orthostatic hypotension.

+ Potent CYP3A4 inhibitors (boceprevir, clarithromycin, cobicistat, erythromycin, itraconazole, ketoconazole, nelfinavir, posaconazole, ritonavir, telaprevir, telithromycin, nefazodone and voriconazole)

Risk of increased plasma concentrations of alfuzosin and its undesirable effects. (See section Special warnings and precautions for use).

Combination requiring precautions for use

+ Phosphodiesterase type 5 inhibitors (avanafil, sildenafil, tadalafil, vardenafil)

Risk of orthostatic hypotension, especially in elderly subjects. Start treatment at the recommended minimum dosage and gradually adjusted the doses if necessary.

Combinations to be taken into consideration**+ Antihypertensives except alpha- blocker**

Increased hypotensive effect. Higher risk of orthostatic hypotension.

+ Dapoxetine

Risk of increased undesirable effects, particularly vertigo or syncope.

+ Medicinal products which lower blood pressure

Increased risk of hypotension, particularly orthostatic hypotension.

FERTILITY, PREGNANCY AND LACTATION**Pregnancy and breast-feeding**

The therapeutic indication does not apply to women. It is not known whether alfuzosin is safe during pregnancy or whether alfuzosin passes into breast milk.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINES

There are no available data regarding the effect of alfuzosin on the ability to drive. Particular caution should be exercised by divers and machines operators because of the risk of orthostatic hypotension, dizziness, asthenia and visual disorders, especially at the start of treatment with alfuzosin.

UNDESIRABLE EFFECTS

The undesirable effects are classified by frequency of appearance using the following convention: very common ($\geq 1/10$); common ($\geq 1/100$, $< 1/10$); uncommon ($\geq 1/1\ 000$, $< 1/100$); rare ($\geq 1/10\ 000$, $< 1/1\ 000$); very rare ($< 1/10\ 000$); not known (cannot be estimated based on available data).

	INCIDENCE			
SYSTEM ORGAN	Common ($\geq 1\% - < 10\%$)	Uncommon ($\geq 0.1\% - < 10\%$)	Very rare ($< 0.1\%$)	Unknown
Cardiac disorder		Tachycardia, palpitations	Angina pectoris in patients with a history of coronary artery disease	Atrial fibrillation
Eye disorders				Intraoperative floppyiris syndrome
General disorder and administration site conditions	Asthenia, malaise	Oedema, chest pain		
Gastrointestinal disorders	Nausea, abdominal pain	Diarrhea, oral dryness, vomiting		
Hepatobiliary disorder				Hepatocellular damage, hepatic cholestasis

Nervous system disorder	dizziness, light headedness, headaches	Syncope, vertigo, smnolence		Ischaemic stroke in patients with underlying cerebrovascular disorders
Reproductive system and breast disorder				Priapism
Respiratory, thoracic and mediastinal disorders		Nasal congestion		
Skin and subcutaneous tissue disorders		Skin eruptions, pruritus	Urticarial, angioedema	
Vascular disorders		Orthostatic Hypotension (see section Special warnings and precautions for use), flushing		
Blood and lymphatic system disorders				Neutropenia, thrombocytopenia

OVERDOSE

In the event of overdose, patient should be hospitalized and kept in the supine position. Standard treatment of hypotension should be initiated. In the event of severe hypotension occurs, a vasoconstrictor which acts directly on blood vessel muscle fibers may be used. Due to its highly protein binding, alfuzosin is difficult to dialyse.

PHARMACOLOGICAL PROPERTIES

Pharmacodynamic properties

Pharmacotherapeutic group:

ALPHA-BLOCKER. ATC code: G04CA01 - G: genitourinary system and sex hormones)

Alfuzosin is an orally active derivative of quinazoline. It is a selective antagonist of post-synaptic alpha-1 adrenergic receptors. In vitro pharmacology studies confirmed the selectivity of alfuzosin for alpha-1 adrenergic receptors located in the prostate, trigone of the urinary bladder and urethra.

Alpha-blockers decrease infra-vesical obstruction by direct action on the smooth muscle of the prostate tissue. In vivo studies in animals have shown that alfuzosin decreases urethral pressure and thus resistance to urinary flow during micturition. A study in conscious rats showed the effect on urethral pressure was greater than the effect on blood pressure.

In placebo-controlled studies in patients suffering from benign prostatic hypertrophy, alfuzosin:

- significantly increases urinary flow by an average of 30% in patients with a flow rate of ≤ 15 ml/s.

This improvement was observed from the first dose,

- significantly decreased detrusor pressure and increased volume causing the need to urinate sensation,
- significantly reduced residual urinary volume.

These effects lead to an improvement in irritative and obstructive urinary symptoms. They did not lead to any harmful effects on sexual function.

Furthermore, the maximum urinary flow rate remained significantly increased 24 hours after administration. In the ALFAUR study, the effect of alfuzosin on the resumption of micturition was evaluated in 357 men over 50 years of age with a first painful episode of acute urinary retention (AUR) related to benign prostatic hypertrophy (BPH) with a voiding residue of between 500 and 1500 ml when the catheter was introduced and during the first hour afterwards. In this randomized, double-blind, parallel-group, multicentre study comparing 10mg/day of alfuzosin LP with placebo, the resumption of micturition was evaluated 24 hours after removal of the catheter, in the morning, after at least two days of treatment with alfuzosin.

Treatment with alfuzosin significantly increased ($p = 0.012$) the rate of resumption of micturition after removal of the catheter in patients who had a first episode of AUR, i.e. 146 patients resumptions of micturition (61.9%) in the alfuzosin group versus 58 (47.9%) in the placebo group.

Pharmacokinetic properties Alfuzosin

The plasma protein binding of alfuzosin hydrochloride is approximately 90%. Alfuzosin undergoes significant metabolism by the liver with only 11% of the unchanged compound parent in the urine. Most of the metabolites (which are inactive) are excreted in faeces (75 to 90%). The pharmacokinetic profile of alfuzosin is unchanged in patients with chronic heart failure.

Prolonged release formulation

After administration of the 10 mg dose to healthy middle-aged volunteers, the mean value of the relative bioavailability is 104.4% compared to that of the immediate-release formulation at a dosage of 7.5mg (2.5mg 3 times daily). Peak plasma concentration is reached 9 hours after administration as compared to 1 hour for the immediate-release formulation. The apparent elimination half-life is 9.1 hours.

Studies have shown that the bioavailability is increased when the drug is administered after a meal (see Posology and method of administration).

Pharmacokinetic parameters (C_{max} and SSC) are not increased in elderly compared to healthy middle-aged volunteers. The mean C_{max} and SSC values are moderately increased in patients with moderate renal failure (creatinine clearance > 30 ml/min), with no modification of the elimination half-life, compared to patients with normal kidney function. Dosage adjustment is, therefore not necessary in patients with renal failure with creatinine clearance of > 30 ml/min.

PHARMACEUTICAL PARTICULARS

List of excipients

Hypromellose, hydrogenated castor oil, ethylcellulose, yellow iron oxide, colloidal hydrated silica, magnesium stearate, mannitol, povidone, microcrystalline cellulose.

Shelf-life: 3 years

Special precautions for storage:

Store below 30°C.

Nature and content of container:

Thermoformed blister packs (PVC/aluminium)

Presentation: 30 tablets

Manufacturer

Sanofi Winthrop Industrie, 30-36,
Avenue Gustave Eiffel,
37100 Tours, France.

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August 2025

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